

[KING FAHAD CENTRAL HOS / STROKE CENTER LETTERHEAD]

Acute Stroke Assessment & Certification Record

Standardized Stroke Assessment Form — Aligned with AHA/ASA Get With The Guidelines-Stroke, Target: Stroke, and Joint Commission / DNV GL Stroke Center Certification Core Measures

STROKE ALERT

Activation Time: _____

Certified Examiner: ☐ Yes ☐ No

Form completed by: _____

1 Patient & Encounter Identification

Patient Name		MRN		Date of Birth		Sex	
Date		Time (24 hr.)		Arrival Mode		Accompanied by	

Document Code: SU-006-FORM-03 | Parent Policy: SU-006 – NIHSS Assessment, Neurological Monitoring and Dysphagia/Swallow Screening | Master Manual: SU-MANUAL-01 | Version: 1.0 | Controlled Copy

2 Symptom Onset & Prehospital Screening

BE-FAST screening tool — record all applicable

Last Known Well (date/time)		Symptom Discovery Time	
Time Patient Found with Symptoms		Witnessed Onset? ☐ Yes ☐ No	
☐ Balance — sudden loss of balance/coordination	☐ Eyes — sudden vision loss/double vision	☐ Face drooping	☐ Arm weakness/drift
☐ Speech difficulty			

3 Stroke Workflow Time Targets

Target: Stroke goal — door-to-needle ≤ 60 min (≤ 45 min premium tier); door-to-groin puncture ≤ 90 min

#	Milestone	Date / Time
1	EMS Notified / 997 Call	
2	EMS On-Scene Arrival	
3	EMS Departure From Scene	
4	Hospital (Door) Arrival	
5	Physician / Provider Exam Start	
6	Stroke Team / Neurology Notified	
7	CT/CTA Order Placed	
8	CT Scan Begin	
9	CT Result Interpreted	
10	Labs Resulted (incl. glucose, INR, platelets)	
11	IV Thrombolytic Decision Made	
12	IV Thrombolytic (tPA/TNK) Bolus — Needle Time	
13	Neurointerventional Team Notified (if LVO)	
14	Groin Puncture Time (if thrombectomy)	
15	Reperfusion / Recanalization Time	
16	Admission to Stroke Unit / ICU	

Calculated Intervals: Door-to-Needle: _____ Door-to-Groin Puncture: _____ Onset-to-Needle: _____ Door-to-CT: _____

4 Vital Signs, Point-of-Care Glucose & Key Labs

Blood Pressure		Heart Rate		SpO ₂	
Temp		Resp. Rate		Finger-stick Glucose	
INR / PT-PTT		Platelet Count		Anticoagulant Use	

5 NIH Stroke Scale (NIHSS)

To be administered by an NIHSS-certified examiner within 12 hours of admission, per Joint Commission Primary Stroke Center requirement. Score each item in the order listed; do not go back and change scores.

Item	Category	Scoring Guide	Score
1a	Level of Consciousness (LOC)	0 — Alert 1 — Not alert, arousable by minor stimulation 2 — Not alert, requires repeated/painful stimulation 3 — Unresponsive / reflex response only	
1b	LOC Questions (month, age)	0 — Answers both correctly 1 — Answers one correctly 2 — Answers neither correctly	
1c	LOC Commands (open/close eyes, grip/release)	0 — Performs both correctly 1 — Performs one correctly 2 — Performs neither	
2	Best Gaze	0 — Normal 1 — Partial gaze palsy 2 — Forced deviation	
3	Visual Fields	0 — No visual loss 1 — Partial hemianopia 2 — Complete hemianopia 3 — Bilateral hemianopia	
4	Facial Palsy	0 — Normal 1 — Minor paralysis 2 — Partial paralysis 3 — Complete paralysis	
5a	Motor Arm — Left	0 — No drift 1 — Drift 2 — Some effort against gravity 3 — No effort against gravity 4 — No movement 9 — Amputation/joint fusion	
5b	Motor Arm — Right	0 — No drift 1 — Drift 2 — Some effort against gravity 3 — No effort against gravity 4 — No movement 9 — Amputation/joint fusion	
6a	Motor Leg — Left	0 — No drift 1 — Drift 2 — Some effort against gravity 3 — No effort against gravity 4 — No movement 9 — Amputation/joint fusion	
6b	Motor Leg — Right	0 — No drift 1 — Drift 2 — Some effort against gravity 3 — No effort against gravity 4 — No movement 9 — Amputation/joint fusion	
7	Limb Ataxia	0 — Absent 1 — Present in one limb 2 — Present in two limbs 9 — Amputation/joint fusion	
8	Sensory	0 — Normal 1 — Mild-moderate loss 2 — Severe-total loss	
9	Best Language	0 — No aphasia 1 — Mild-moderate aphasia 2 — Severe aphasia 3 — Mute/global aphasia	
10	Dysarthria	0 — Normal 1 — Mild-moderate 2 — Severe 9 — Intubated/Intubated/another barrier	
11	Extinction / Inattention (Neglect)	0 — No abnormality 1 — Mild (one modality) 2 — Severe (more than one modality)	

TOTAL NIHSS SCORE (0–42)

Severity: ☐ 0 None ☐ 1–4 Minor ☐ 5–15 Moderate ☐ 16–20 Mod-Severe ☐ 21–42 Severe

6 Neuroimaging & Stroke Classification

Non-contrast CT Findings		ASPECTS Score	
CTA / LVO Identified		Occlusion Site (if any)	
Stroke Type		Suspected Etiology (TOAST)	
☐ Ischemic	☐ Hemorrhagic	☐ TIA	☐ Undetermined
☐ Stroke Mimic			

7 Thrombolytic (IV tPA/TNK) & Thrombectomy Eligibility Checklist

Confirm against current institutional protocol and AHA/ASA Acute Ischemic Stroke guideline before treatment decision

Inclusion Criteria Confirmed	Exclusion Criteria Screened (None Present)
☐ Acute ischemic stroke with measurable neurologic deficit ☐ Onset (or last known well) within treatment window per current AHA/ASA guideline ☐ Age $\geq$ 18 years ☐ CT/MRI excludes hemorrhage	☐ Active internal bleeding or bleeding diathesis ☐ Platelet count $< 100,000/\text{mm}^3$ ☐ Current anticoagulant use with INR > 1.7 or elevated PT/PTT ☐ Systolic BP > 185 or diastolic BP > 110 despite treatment ☐ Blood glucose < 50 or > 400 mg/dL not corrected ☐ History of intracranial hemorrhage ☐ Recent major surgery, GI/GU hemorrhage, or trauma (per protocol interval) ☐ CT evidence of multilobar infarction (hypodensity $> 1/3$ hemisphere)

Treatment Decision		Weight-based Dose	
Consent Obtained By		Endovascular Consult	
☐ IV Thrombolytic Given	☐ Mechanical Thrombectomy	☐ Neither — Contraindicated	☐ Transferred to Higher Level of Care

8 Disposition, Certification Statement & Sign-Off

Disposition		Unit / Bed	
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By signing below, I attest that this assessment was performed and documented in accordance with the hospital's Stroke Center Certification protocol, consistent with AHA/ASA Get With The Guidelines-Stroke performance measures and applicable Joint Commission / DNV GL primary, thrombectomy-capable, or comprehensive stroke center certification standards. Data elements above are intended for concurrent clinical documentation and retrospective abstraction for stroke registry / core measure reporting.

Examining Clinician (print name / credentials)	Signature	Date / Time
NIHSS Certification # / Expiration	Stroke Team / Neurology Attending	Quality/Registry Abstractor Review

Form Reference: Built for alignment with the AHA/ASA Get With The Guidelines-Stroke registry data elements, the Target: Stroke initiative time benchmarks, and Joint Commission/DNV GL/CARF stroke center certification survey requirements current as of the review date below. Institutions should validate all clinical thresholds, time windows, and inclusion/exclusion criteria against their own medical staff-approved stroke protocol and the current AHA/ASA Guidelines for the Early Management of Patients With Acute Ischemic Stroke prior to clinical use — this template is an administrative documentation aid, not a substitute for institutional protocol or clinical judgment.